

Expert Report on Amendments to the Rights of Persons with Disabilities Rules, 2017: Mandating Accessibility Standards for Rural Water Supply and Sanitation Infrastructure in India

I. Executive Summary: The 2023 Mandate for Universal Accessibility

This report details the profound statutory shift initiated by the Rights of Persons with Disabilities (Amendment) Rules, 2023 (G.S.R. 07(E)), published in the Gazette of India on January 3, 2024. These rules amend the Rights of Persons with Disabilities Rules, 2017, by formally incorporating highly technical and granular guidelines related to essential rural infrastructure—piped water supply (PWS) and community sanitation (Community Sanitary Complexes, or CSCs).¹ The primary implication of this amendment is that technical standards previously issued as administrative directives by the Department of Drinking Water and Sanitation (DDWS) are now enforceable under the Rights of Persons with Disabilities (RPwD) Act, 2016. This mandates immediate compliance and integration of universal design principles across national flagship schemes like the Jal Jeevan Mission and Swachh Bharat Mission (Grameen) Phase II.¹

The key legislative action involves inserting two clauses into Rule 15(1) of the RPwD Rules, 2017, thereby making the DDWS Guidelines for PWS (August 2, 2023) and the CSC Standards (August 4, 2023) mandatory components of accessibility regulation.¹ A crucial feature of the PWS guidelines is the explicit extension of accessible design benefits to those facing "temporary access challenges," such as pregnant women or individuals with short-term ailments, reinforcing a broad universal design philosophy.¹ Furthermore, the CSC standards emphasize pragmatic, durable solutions tailored for rural environments, including clear specifications for accessible pathways (1:15 ramp slope, 1200 mm width), specific hardware

(dual-height handrails, precisely placed grab bars), and locally maintainable infrastructure.¹

II. Legislative Foundation and Regulatory Implementation

A. The Enabling Framework: Section 40 of the RPwD Act, 2016

The legal requirement for these sweeping amendments originates from Section 40 of the Rights of Persons with Disabilities Act, 2016 (49 of 2016).¹ This section mandates the Central Government to formulate rules laying down standards of accessibility for facilities and services provided to the public in both urban and rural areas. This must be executed in consultation with the Chief Commissioner for Persons with Disabilities.¹ The necessity for the government to act based on this mandate provided the foundational link between social justice legislation and national infrastructure policy.

The process detailed in the Gazette confirms that a draft of the amendments was published on September 21, 2023, inviting objections and suggestions, copies of which were made available to the public on September 28, 2023.¹ Following consideration of the feedback received, the final rules were enacted.

B. The Rights of Persons with Disabilities (Amendment) Rules, 2023 (G.S.R. 07(E))

Published on January 3, 2024, and effective immediately upon publication, these Rules amend the Rights of Persons with Disabilities Rules, 2017, by inserting two critical clauses into sub-rule (1) of rule 15, which governs minimum accessibility standards.¹

The regulatory consequence of this action is profound. Previously, the detailed technical specifications issued by the DDWS existed primarily as policy guidelines. By inserting the specific notification numbers of the DDWS documents into the core RPwD Rules, the government has made these granular technical specifications (such as the maximum ramp slope of 1:15, or the precise placement of handrails) subject to the legal accountability

provisions of the RPwD Act.¹ This elevation ensures that implementing bodies, from Central agencies to Panchayati Raj Institutions (PRIs), must achieve compliance with these technical requirements, transforming adherence from voluntary best practice into a statutory obligation.

The specific insertions into Rule 15(1) are summarized below:

Statutory Amendments to RPwD Rules, 2017 (Rule 15(1))

New Clause	Reference Notification (DDWS)	Subject Matter	Date of Notification
(i) [झ]	CDN-11017/58/2021 -Coordination-DD WS	Guidelines on accessible and inclusive piped water supply for persons with disabilities and other population groups with access challenges.	2nd August, 2023 ¹
(j) [ञ]	S-18012/5/2021-SB M-DDWS	Accessibility standards for community toilets in Rural areas.	4th August, 2023 ¹

C. Statutory Inclusion of DDWS Accessibility Standards

The insertion of clauses (i) and (j) officially incorporates the technical standards finalized by the Department of Drinking Water and Sanitation (DDWS) under the Ministry of Jal Shakti.¹ This legislative action creates a strong link between accessibility mandates and two flagship national programs: the provision of Piped Water Supply (PWS), often delivered through the Jal Jeevan Mission (JJM), and the development of Community Sanitary Complexes (CSCs) under the Swachh Bharat Mission (Grameen) Phase II.¹

III. Guidelines on Accessible and Inclusive Piped Water Supply (Notification CDN-11017/58/2021-Coordination-DDWS)

The Guidelines on accessible and inclusive piped water supply aim to provide comprehensive information for designing PWS infrastructure that is usable at the household, community, and institutional levels.¹ The document draws upon established benchmarks, including the Handbook on Barrier Free and Accessibility from the Central Public Works Department (CPWD), the Ministry of Housing and Urban Affairs (MOHUA) Harmonised Guidelines (2021), the National Building Code (NBC), and International Standard ISO 21542: 2011.¹

A. Universal Design Philosophy and Scope

The core objective of these guidelines is to ensure PWS facilities are accessible for persons with disabilities and other population groups facing access challenges.¹ This approach goes beyond the 21 categories of disabilities defined in the RPwD Act. The scope explicitly includes individuals with temporary access challenges, such as

pregnant women, mothers with young children, or persons suffering from short-term physical ailments.¹ By formally recognizing that access limitations can be situational and temporary (e.g., due to injury or pregnancy), the mandate promotes Universal Design principles, ensuring the design benefits the widest possible population and guarantees dignity for every user at all times.¹

The document identifies major disabilities that currently form barriers to accessing piped water, including blindness, low-vision, locomotor disability, dwarfism, cerebral palsy, muscular dystrophy, chronic neurological conditions, Parkinson's disease, and multiple disabilities.¹

B. Functional Requirements of an Accessible PWS Facility

The guidelines strictly define an accessible piped drinking water facility based on four

mandatory functional criteria ¹:

Functional Requirements Defining an Accessible PWS Facility

Requirement Category	Criterion for Accessibility
1. Barrier Resolution	Addresses barriers to reaching the tap, entering/exiting the space, and using the tap to source water, including space for carrying water.
2. Safety	Ensures the safety of the users at all times.
3. Ease of Access	Ensures ease of access to all users, especially PwDs and the elderly, at all times.
4. Dignity	Ensures the dignity of the users at all times.

The criterion of "Barrier Resolution" is critical, focusing not just on reaching the tap but also on accommodating water usage needs, specifically providing space to hold containers or bottles without continuous strain.¹

C. Engineering and Design Principles for PWS Points

The design considerations prioritize longevity, functionality, and local viability. In situations where PWS points already exist in institutional or community facilities, **retrofitting** is prioritized over new installations.¹ Retrofitting typically involves simple, effective adjustments such as changing tap types, modifying water point heights, adding support bars, and introducing clear signs or markers.¹

Key general design considerations include:

1. **Height Adjustments:** The water point height must be flexible. For children, the height should be between 500 mm and 700 mm. For wheelchair users, the height must be less than 850 mm. This necessitates setting up either two water points at different heights or using a flexible modular unit.¹

2. **Tap Operation:** Taps should be easy to use for all persons. They must be operable by one hand, requiring no tight grasping, pinching, or twisting of the wrist. Lever-type handles, foot-pedal operated mechanisms, or automatic sensors are preferred.¹ The force required to activate the control must be minimal, specified as not greater than 22 N (CPWD/HG) or less than 20 N (NBC).¹
3. **Durability and Maintenance:** Materials selected for outdoor use must be robust, such as stainless steel, to prevent corrosion, or UV-resistant plastics. Designs must be strong to ensure a long-term facility with minimal maintenance required.¹ The principle is that the long-lasting facility provides better value, requiring consideration of both capital cost and ongoing operation and maintenance (O&M) over the design life.¹

D. Detailed Technical Dimensions for Water Points and Wash Basins

Specific technical standards are mandated for wash basins, which are common at PWS points:

- **Wash Basin Height:** The rim or top edge should be between 750 mm to 850 mm from the floor level. The Harmonized Guidelines (2021) suggest one height between 750 mm and 800 mm for wheelchair users and another at 900 mm for standing adults.¹
- **Knee and Toe Clearance:** Crucially, unobstructed space must be maintained beneath the wash basin for wheelchair users. This knee space must be at least 750 mm wide, 200 mm deep, and 650 mm to 700 mm high.¹
- **Maneuvering Space:** A minimum clear floor space of 900 mm wide by 1200 mm deep is required in front of the wash basin, although a maximum depth of 480 mm may be accommodated under the basin itself.¹
- **Tap Reach:** The distance to the tap control must be a maximum of 300 mm, with the tap located within 350 mm to 600 mm of the wall.¹

E. Solutions for Specific Impairments in PWS

The guidelines provide targeted interventions based on the type of barrier¹:

1. **Locomotor Disability (Mobility Impairments):** Pathways leading to water points must be constructed of non-slip, non-uneven materials, with contrasting colors used to identify changes in surface level (e.g., before and after steps). Handrails or grab bars are necessary along the path for support and on both sides of the water point.¹ Adequate space must be provided around the water point for a wheelchair to enter and turn

comfortably.¹

2. **Vision Impairments (Blindness, Low Vision):** Accessibility for visually impaired persons requires integrating sensory cues. This includes providing **bold signs, clear designation of places, warnings, and routing information**.¹ Simple wayfinding solutions like landmark posts or guide strings are recommended along the path to the tap.¹ To aid orientation, the area around the tap must be well illuminated, and adequate **color or tonal contrast** must be applied between walls, floors, and fixtures.¹ Addressing the challenge of water collection, the guidelines suggest innovative sensory feedback mechanisms, such as **taps with voice sensors or audio indicators** to signal when the tap is turned on/off or when a container is full, along with Braille indicators.¹
3. **Institutional Settings:** Facilities such as Anganwadi Centers, schools, and health centers must provide a minimum of two common water points. Residential complexes must provide at least one accessible drinking water point per floor in dormitory, staff, and visitor areas.¹ Health centers (PHC, CHC, hospitals) must provide at least one accessible point per ward (male, female, pediatric).¹

IV. Accessibility Standards for Community Toilets in Rural Areas (Notification S-18012/5/2021-SBM-DDWS)

These standards focus on ensuring that Community Sanitary Complexes (CSCs)—a key element of rural sanitation—are fully inclusive.¹ Recognizing the operational realities of rural infrastructure, the document advises against complex technology-intensive solutions, such as sensor-based doors or dustbins, due to their high capital costs and the difficulty of ensuring regular O&M in community-operated rural areas.¹ The focus is placed instead on reliable, low-tech hardware and precise dimensional compliance.

A. Design Solutions by Disability Type

The design solutions are categorized by the difficulties they address ¹:

- **Physical Disability/Mobility Impairments:** Require smooth, barrier-free paths; ramps with adequate slope; non-slippery floors; adequate maneuvering space within the cubicle; raised seats; and specific grab bars.¹
- **Vision Impairment:** Requires guide strings along the path, well-illuminated areas, and

sufficient color/tonal contrast between walls, floors, and fixtures.¹

- **Speech and Hearing Impairment:** Requires the use of proper, clear signage that includes both symbols and text (using a bold, 18 pt font) placed at a height of 1500 mm to 1800 mm from the ground.¹

B. External Access and Pathway Compliance

Stringent adherence to dimensional specifications for access routes is required for external compliance ¹:

- **Access Pathway:** Must be clear, smooth, and free of obstacles, constructed of antiskid floor material, and using tactile tile fixing as specified. The pathway must maintain a minimum clear width of 1200 mm.¹
- **Ramp Slope and Landing:** The entrance must include a ramp with a moderate slope, specifically set at a maximum gradient of **1:15** (one unit vertical rise for every 15 units horizontal run). The landing area before the ramp must provide at least 1200 mm×1200 mm of clear space.¹
- **Dual Handrails:** Ramps must be equipped with continuous handrails on both sides at two levels: the lower rail fixed at 700 mm to 750 mm height, and the upper rail fixed at 850 mm to 900 mm height from the finished floor. The ends must be rounded, grouted, and extend 300 mm beyond the ramp ends.¹
- **Corridor Width:** Inside the building, corridors must maintain a minimum clear passage width of 1200 mm to accommodate wheelchair access.¹
- **Steps:** Accessible steps must have a uniform riser (maximum 150 mm) and tread (minimum 300 mm) with an unobstructed width of at least 1200 mm. They must also have continuous two-level handrails mirroring the ramp specifications.¹

C. Accessible Toilet Cubicle Specifications

The accessible cubicle is central to the CSC design and features precise requirements to ensure safe transfer and maneuvering.¹

- **Internal Dimensions:** The inner space should preferably be 1.80 m×2.00 m. Crucially, there must be a clear space of **1200 mm in front of the Water Closet (WC)** to allow for easy circular movement of a wheelchair.¹
- **Door Specifications:** The door must have a 900 mm clear width and must open **outward** for safety and convenience, although a sliding door is also advisable. The

handle must be D-shaped and mounted horizontally at 850 mm to 1000 mm from the floor.¹

- **WC Type and Height:** The WC must be of European style (Commode), wall-mounted, and installed precisely at **0.48 meters** above the floor level. This specific height is mandated to align with standard wheelchair seat heights, thereby facilitating safer and more dignified transfer.¹ An alarm bell must also be included for emergency purposes.¹
- **Grab Bar Configuration:** The placement of grab bars is detailed to support transfer maneuvers. An **L-shaped grab bar** (700 mm×700 mm) must be mounted at 0.7 meters high on the wall side. A **U-shaped grab bar** must be mounted at 480 mm high on the transfer side.¹ The dimensional precision of these bars is essential for the biomechanically optimized movement of wheelchair users. In the rural context, GI pipe or stainless steel bars (25 mm to 35 mm diameter) fixed at 700 mm from the ground on both sides are acceptable alternatives.¹
- **Urinals:** Urinal locations must be step-free. At least one urinal should be fixed at a lower height and include grab bars on each side and in front to support ambulant persons, such as crutch users.¹

D. Wash Basin and Sensory Requirements within the CSC

Within the CSC, wash basins and fixtures must also adhere to accessibility principles.¹

- **Wash Basin Height and Clearance:** The wash basin top edge must be between 700 mm and 800 mm from the floor, providing a clear knee space of at least 760 mm wide by 200 mm deep and 650 mm to 680 mm high.¹ The wash basin itself must contrast with the wall color.¹
- **Taps:** Taps must be of the long lever type, preferably fixed at 0.7 to 0.8 meters from the floor level.¹
- **Sensory Cues:** Signage must be clearly visible, in contrasting colors, and supplemented with **Braille facility**.¹ Contrast color banding around the toilet door and on inner walls is required for low vision users, and fixtures must contrast with the inner wall color.¹

V. Policy Implications, Implementation Challenges, and Recommendations

The amendments represent a critical legal mandate, merging social justice goals with

infrastructure development. However, implementation hinges on overcoming institutional and logistical challenges inherent to decentralized rural governance.

A. Institutional and Capacity Gaps

The detailed nature of the standards—such as the requirement for a 1:15 ramp gradient or the specific heights for grab bars (0.7 m and 0.48 m)—necessitates a significant level of technical precision.¹ The document itself identifies existing systemic weaknesses, including the exclusion of persons with disabilities from the planning process, the lack of clear policy guidelines for accessible PWS, and a widespread absence of technical capacity and awareness materials (IEC) among key stakeholders, including PRIs and local service providers.¹

Achieving full compliance requires that local government bodies and the technicians they employ move beyond simple provision (e.g., "a ramp is present") to functional compliance (e.g., "the ramp meets the 1:15 slope requirement, and the handrail is at the precise dual height"). This implies a substantial disconnect between the high-level regulatory mandate and the grassroots technical capability required to execute it.¹

B. Recommendations for Sustainable Implementation

To successfully translate these statutory requirements into tangible, accessible rural infrastructure, several focused actions are required:

1. **Prioritized Technical Capacity Building:** A national program for technical capacity building and IEC must be immediately launched. This training must specifically target masons, plumbers, local engineers, and PRI leaders, instructing them on the exact dimensional requirements for accessible PWS points and CSC cubicles, focusing on features like knee clearance, WC transfer height, and required contrast application.¹
2. **Mandatory Accessibility Audits:** Compliance monitoring must shift from general inspections to objective, dimensional audits. Audits should verify the functional accessibility of the facilities by measuring elements like ramp gradients, WC heights (0.48 m), and the configuration and strength of grab bars, ensuring that the infrastructure supports safe and dignified use as intended.¹
3. **Sustainable Operations and Maintenance (O&M):** Given the emphasis on durability and local repairability, funding frameworks for PWS and CSC projects must include a mandated, dedicated O&M component. Deterioration of critical features (e.g., non-slip surfaces becoming slippery, tap mechanisms failing, or contrast paint fading) renders the

facility inaccessible, regardless of its initial compliant construction.¹ Establishing robust local supply chains for appropriate spare parts is crucial to ensure long-term functionality.¹

VI. Conclusion: The Promise of Inclusive Rural Development

The Rights of Persons with Disabilities (Amendment) Rules, 2023, mark a pivotal moment by legally cementing the requirement for fully accessible and inclusive Piped Water Supply and Community Sanitary Complexes in rural India.¹ This legislative action aligns social justice mandates with national infrastructure priorities, demanding that flagship schemes like JJM and SBM-G deliver services that uphold the dignity and right to access for all citizens, including those with temporary or permanent access challenges. The success of this mandate will depend on the effectiveness of implementation and the commitment of local governance structures to acquire the necessary technical capacity to build and sustain these precisely engineered accessible facilities.

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